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Review

IOIBD: the International Organization for the Study of Inflammatory Bowel Disease

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***Abstract*:** The International Organization for the Study of Inflammatory Bowel Disease (IOIBD) is an international scientific organization that has shaped the framework for inflammatory bowel disease (IBD) research, clinical management strategy, therapeutic development, and clinical trial methodology for more than four decades. Formally constituted in April 1981 in Lyon, France, IOIBD was created to address fundamental barriers to scientific and clinical progress in IBD, including inconsistent definitions of disease activity and outcomes across studies and countries. Since the establishment of the IOIBD Foundation for Research and Education in 1997, IOIBD has combined a highly engaged global membership of experts with structured governance, continuously active thematic clusters, and an annual rotating international meeting to deliver consensus frameworks and collaborative initiatives that translate directly to clinical practice and regulatory and translational science. Key outputs include studies of global epidemiology of IBD, the Selecting Therapeutic Targets in IBD (Selecting Therapeutic Targets in Inflammatory Bowel Disease, STRIDE) treat-to-target programs; the SPIRIT consensus initiative addressing long-term disease impact and endpoints for disease-modification trials; validated approaches to capturing disability and patient-reported outcomes; consensus guidance on nutrition and diet as modifiable and potentially disease-modifying factors; recommendations to optimize IBD clinical trial design and endpoints; reclassification of IBD initiative; rapid international guidance during the COVID-19 pandemic; and educational initiatives including topic-focused satellite symposia, the Helmsley–IOIBD Clinical Experience Exchange Program, and the Empowering Women in IBD Leadership Program (EMPOWHER). This manuscript reviews IOIBD's history, operational model, selected scientific contributions, educational mission, and evolving strategy as the field moves toward precision medicine, globalization of care, and data-intensive approaches, including artificial intelligence.

Plain language summary

International organization for the study of inflammatory bowel diseases (IOIBD)

Inflammatory bowel disease (IBD), including Crohn's disease and ulcerative colitis, faces ongoing challenges. Although treatment has improved, many patients still face delayed diagnosis, ongoing inflammation, complications, surgery, and reduced quality of life. These challenges have also made it hard for researchers and clinicians worldwide to compare

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studies and agree on the best ways to measure improvement. This article reviews the work of International Organization for the Study of Inflammatory Bowel Disease (IOIBD), an international network of experts founded in 1981 to help solve these problems through collaboration and shared standards. IOIBD brings together clinicians and scientists from many countries and disciplines, works year-round in focused topic groups, and meets annually to develop practical guidance and shared definitions that can be used in research studies, clinical trials, and everyday care.

Keywords: clusters, education, expert consensus, IBD care, International collaboration, IOIBD, research, treat to target

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Introduction

Despite major advances in the field of inflammatory bowel diseases (IBD), important unmet needs remain: diagnostic delays, refractory inflammation, treatment intolerance, complications, recurrent hospitalizations and surgery, impaired quality of life, long-term disability, and disparities. The rising global incidence and prevalence of IBD—particularly in newly industrialized regions—adds urgency to defining scalable, evidence-based models for care and harmonized methodologies for research.[1]

Addressing these challenges requires sustained international collaboration that can integrate evidence-based medicine, epidemiology, trial methodology, translational science, patient-reported outcomes, definition of new endpoints as well as disease monitoring strategies. IOIBD was conceived as a focused forum designed to resolve foundational problems in disease assessment, trial design, and clinical management. Its model—selective membership, year-round scientific clusters, and consensus-driven outputs—has enabled IOIBD to align the field around practical targets and standardized approaches, while remaining responsive to emerging scientific and societal realities.

IOIBD occupies a distinct niche within the global gastroenterology landscape. Unlike large open-membership societies such as the European Crohn's and Colitis Organisation, the United European Gastroenterology, or the American Gastroenterological Association or the American College of Gastroenterology—which serve broad clinical communities through guidelines, educational meetings, and advocacy—IOIBD functions as a compact, invitation-only global think tank. Its membership is currently capped, including experts spanning multiple disciplines and more than 25 countries, and is intentionally designed to foster accountability, sustained engagement, and rapid consensus. Rather than hosting large-scale open conferences or producing comprehensive clinical practice guidelines, IOIBD focuses on foundational infrastructure: it defines the research agenda, standardizes trial endpoints, and develops the conceptual and methodological frameworks that inform and underpin the work of broader organizations and regulatory agencies—all through considerations that are global in scope. This Agile model enables IOIBD to move efficiently from scientific question to actionable consensus, making it uniquely positioned to interface with regulatory bodies, industry partners, and the wider gastroenterology community as an authoritative and independent source of direction in IBD research and care.

History and evolution

IOIBD was formally constituted at a meeting in Lyon in April 1981, hosted by Professor Louis Descos and Dr Claude André, following years of discussion focused on improving the assessment of Crohn's disease activity.[2] The organization emerged at a time when disparate indices and definitions limited meaningful comparison across studies and impeded drug development. From its inception, IOIBD adopted a membership model that prioritized active participation and rigorous debate.

In 1997, IOIBD established a formal Foundation for Research and Education, legally registered in Sweden as Stiftelsen för utbildning och forskning

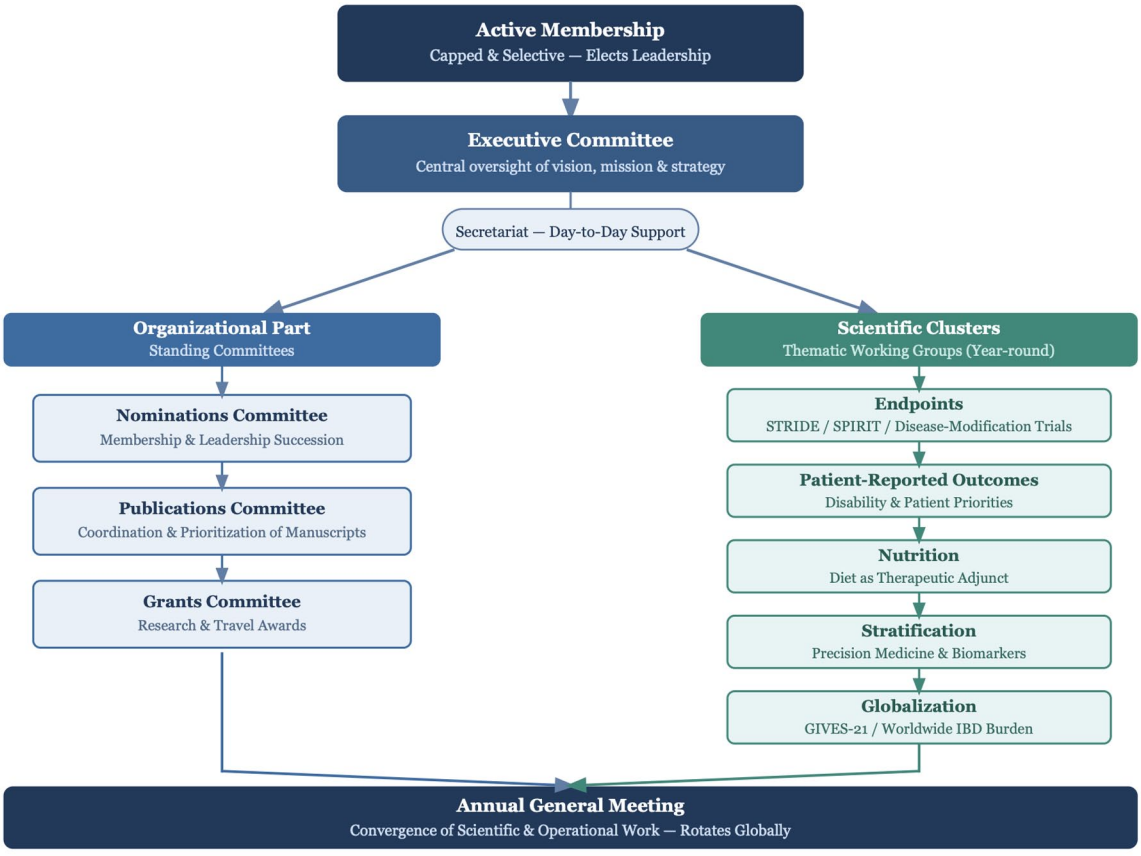


Figure 1. Schematic overview of IOIBD governance and activity, including membership, executive leadership, standing committees, year-round scientific clusters, and the Annual General Meeting where work converges. IOIBD, International Organization for the Study of Inflammatory Bowel Disease.

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avseende inflammatoriska tarmsjukdomar, enabling an expanded commitment to non-commercial research and educational initiatives while preserving academic independence.[1] Over subsequent decades, IOIBD's scope broadened from activity indices to a mission focused on promoting the health of people with IBD worldwide by setting direction for patient care, education, and research, with particular attention to countries where IBD is evolving. This evolution reflects the shifting landscape of IBD therapeutics and outcomes science, including objective inflammatory targets, patient-reported outcomes and disability, nutrition and microbiome science, and the globalization of IBD epidemiology and care.

Structure and operations

IOIBD operates under defined bylaws with a selective, capped membership of including experts representing more than 25 countries and spanning adult and pediatric gastroenterology, surgery, epidemiology, pathology, trial design, biostatistics, and translational science. The limited size supports high-engagement participation and accountability for deliverables. IOIBD organizational structure is further detailed in Figure 1.

Scientific activity is currently organized through five continuously active thematic clusters: (1) Endpoints, (2) Patient-Reported Outcomes, (3) Nutrition, (4) Stratification, and (5) Globalization. Cluster work proceeds throughout the year and culminates in presentations and consensus deliberations at the Annual General Meeting, which rotates globally.[2]

IOIBD's vision and mission are overseen by an Executive Committee and supported by standing committees, including a Nominations Committee (membership and leadership succession), a Grants Committee (review and support of

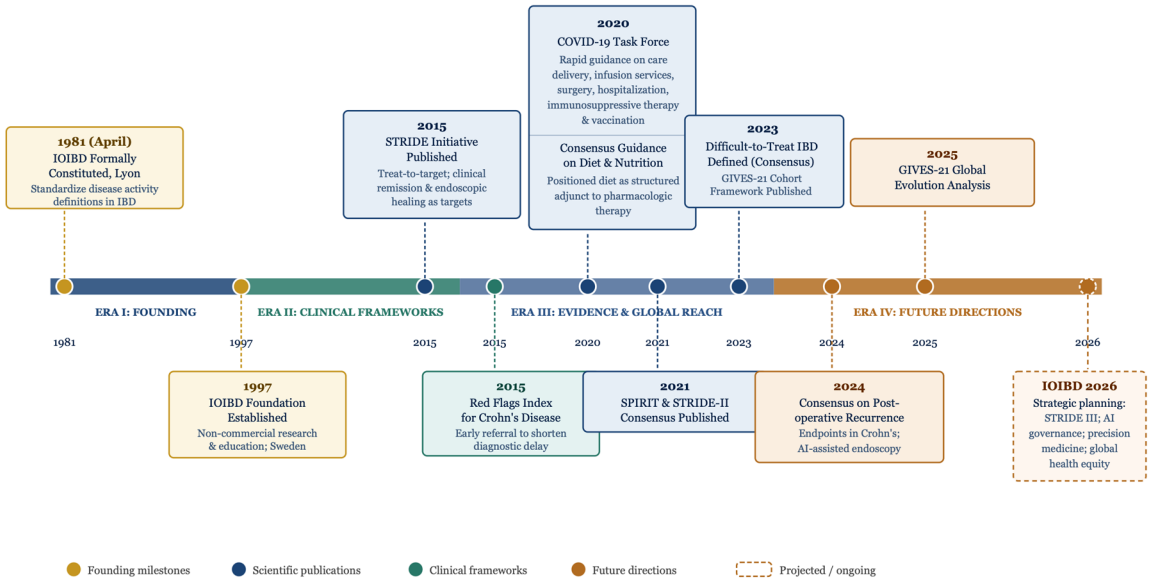


Figure 2. Key milestones across IOIBD's evolution (1981–2030), highlighting foundational steps, major consensus initiatives, global activities, and forward-looking priorities.
IOIBD, International Organization for the Study of Inflammatory Bowel Disease.

innovative projects), and a Publications Committee (prioritization and coordination of IOIBD manuscripts). To preserve academic independence while supporting operational sustainability, IOIBD activities are supported by the IOIBD Foundation, alongside the IOIBD Institute (a Dutch-registered entity) that helps obtain third-party financing for non-profit activities and organizes educational programs, including topic-focused satellite symposia, within transparent governance frameworks.

Corporate members from the biopharmaceutical industry participate in IOIBD through a fee-based program and scientific discussions but do not take part in organizational decisions or the Annual General Business Meeting, reflecting a framework intended to promote translational dialogue and implementation of IOIBD insights while safeguarding academic autonomy.

Scientific output and impact

IOIBD's scientific output is characterized by consensus-driven frameworks that translate directly to clinical practice, trial methodology, and regulatory science. A defining feature is the focus on actionable targets and standardized definitions that can be adopted across settings and that have a significant impact on patient management and therapeutic development. The major milestones and initiatives of IOIBD are summarized in Figure 2.

Treat-to-target and therapeutic goals (The Selecting Therapeutic Targets in Inflammatory Bowel Disease)

The Selecting Therapeutic Targets in Inflammatory Bowel Disease (STRIDE) initiative, launched by IOIBD, established treat-to-target strategies for IBD by defining long-term goals beyond symptom control, including clinical remission and endoscopic healing.[3] STRIDE II updated these targets to time-dependent targets and included the additional targets biomarker normalization, restoration of quality of life, growth normalization in pediatric patients, and the absence of disability, while positioning histologic and transmural healing as important aspirational targets.[4] Collectively, STRIDE frameworks have become foundational to trial design and have accelerated adoption of objective monitoring and therapeutic adjustment in routine care. As further prospective data are accumulating, IOIBD is currently updating STRIDE and plans to present the STRIDE III consensus statement within the coming year.

Long-term impact and disease-modification endpoints (The Selecting Endpoints for Disease-Modification Trials)

IOIBD recognized that sustained benefit requires addressing the long-term impact of IBD on patients' lives, including function, participation, work productivity, and cumulative disease burden. The Selecting Endpoints for Disease-Modification Trials (SPIRIT) consensus, convened under the auspices of IOIBD, proposed endpoints for trials designed to prevent disease progression, including medium-term outcomes (hospitalization and surgery) and longer-term risks (cancer and mortality), along with patient-centered impact (health-related quality of life, disability).[5] This work has provided a common language for disease-modification trials and encouraged validation of endpoints that capture both biology and lived experience.

Patient-reported outcomes and disability

IOIBD has supported the development and validation of disability-focused instruments, recognizing disability as both an outcome and a target.[6] Integrating disability and other patient-reported outcomes into trials and care pathways encourages more comprehensive assessment of therapeutic benefit and aligns with patient priorities.

Nutrition and potentially disease-modifying strategies

IOIBD consensus guidance on diet and nutrition emphasized the role of dietary patterns and food components in influencing inflammation and symptoms, and positioned diet as a structured adjunct to pharmacologic therapy.[7] By articulating pragmatic dietary guidance and research priorities, IOIBD helped legitimize nutrition as a core pillar of IBD care and stimulated trials integrating diet, microbiome-directed approaches, and patient-centered outcomes.

Reducing diagnostic delay: Red flags for Crohn's disease

IOIBD initiatives have also highlighted the importance of earlier recognition of IBD, including work on "red flags" indices intended to shorten diagnostic delay in Crohn's disease,[8] with the goal of enabling earlier intervention before irreversible bowel damage occurs.

Difficult-to-treat IBD

The concept has gained prominence as therapeutic options have expanded, revealing a subset of patients who remain symptomatic or active despite multiple advanced therapies. IOIBD-supported work has helped move this concept from an imprecise label to a research-ready construct. An international expert effort defined difficult-to-treat IBD as failure of biologics and advanced small molecules with at least two different mechanisms of action and further recognized clinical scenarios that commonly drive high burden and complexity, including chronic antibiotic-refractory pouchitis, complex perianal Crohn's disease, postoperative recurrence after repeated resections, and psychosocial comorbidities that impair effective disease management.[9] Clinically, the value of a shared definition is that it reframes refractoriness as multidimensional and enables focused research.[10] Persistent symptoms may reflect ongoing inflammation, but may also arise from structural damage, altered motility, bile acid diarrhea, overlapping functional disorders, malabsorption, infection, or medication-related adverse effects. In addition, pharmacokinetic failure (subtherapeutic exposure), immunogenicity, and mechanistic mismatch can mimic primary non-response. Psychosocial challenges, health literacy, treatment fatigue, and access barriers can also limit effective implementation of care.

Clinical trial design, endpoint harmonization, and regulatory interface

IOIBD has served as a high-credibility interface between academia, industry, and regulators. Consensus outputs have addressed eligibility criteria, endpoint selection, index choice, washout periods, recruitment strategies, integration of biomarkers and patient-reported outcomes,[11,12] and management of postoperative recurrence following ileocolic resection in Crohn's disease.[13]

These efforts improve interpretability across trials and support regulatory dialogue by aligning endpoints with clinically meaningful benefit.

Stratification and reclassification toward precision medicine

IOIBD has contributed to the shift from syndrome-based labels toward biologically informed stratification, integrating genetics, immune pathways,

disease location/behavior, and patterns of therapeutic response. These efforts support trial enrichment strategies and a future in which treatment selection is increasingly mechanism-based. IOIBD's contribution is to encourage a standardized, stepwise approach: confirm and phenotype active inflammation; define predominant drivers of symptoms; optimize prior therapies using exposure-response principles; incorporate multidisciplinary care (nutrition, mental health, surgery, radiology); and rationally sequence or combine therapies based on mechanism and risk. For trial methodology, consensus definitions facilitate enrichment and comparability across studies, enabling evaluation of emerging approaches—novel mechanisms, combination strategies, microbiome-directed interventions, and data-informed (including AI-assisted) treatment selection—in populations with the highest unmet need.

Globalization and epidemiology (Global IBD Visualization of Epidemiology Studies in the 21st Century)

Through its Globalization cluster, IOIBD has strengthened its focus on the worldwide burden of IBD and the need for harmonized population-based data in newly industrialized regions. The Global IBD Visualization of Epidemiology Studies in the 21st Century (GIVES-21) inception cohort framework reflects IOIBD's commitment to standardized, prospective capture of incidence and outcomes across diverse health-care systems, enabling comparisons and guiding resource planning and research prioritization.[14] In a landmark study, the GIVES-21 consortium analyzed over 500 population-based epidemiologic studies spanning 80 regions and nearly a century of data to establish the evolution of IBD across four epidemiologic stages.[15]

Shaping a global research agenda for better management and public health response

Regional disparities in health-care access contribute to suboptimal disease management in certain regions. Addressing these inequities is crucial for improving global IBD outcomes. IOIBD convened experts from clinical care, research, public health and advocacy (including patient representation) and identified priorities in epidemiology, care models, treatment strategies, education and awareness, patient and community engagement, and leadership to promote health equity. These priorities emphasize quantifying the burden of IBD, addressing health-care disparities, validating care models, exploring novel treatments, advancing education, engaging patients, and advocating for health equity policies. The identified priorities may serve as a guide for both clinical and non-clinical researchers, and funders dedicated to IBD-related initiatives, fostering international collaboration to improve IBD management and reduce its impact globally.[16–18]

Rapid global response during the COVID-19 pandemic

Evidence of the globalization and agility of IOIBD was demonstrated during the COVID-19 pandemic. IOIBD convened an international task force to provide guidance on care delivery, infusion services, surgery, hospitalization, immunosuppressive therapy, and vaccination. These outputs demonstrated the organization's capacity to synthesize evidence and expertise quickly to support clinicians worldwide during periods of uncertainty.[19–21]

Education and training activities

Education and mentorship are integral to IOIBD's mission to set direction for patient care, education, and research worldwide. Educational activities are designed to translate science into practical frameworks, expand global capacity, and cultivate future leaders.

Scientific and educational grants: each year, grants are awarded to four clinical and basic studies, selected based on scientific merit. Two travel grants are provided as well.[2]

Helmsley–IOIBD clinical experience exchange program

To strengthen global IBD capacity—particularly in low-income nations—IOIBD partnered with the Leona M. and Harry B. Helmsley Charitable Trust to create a Clinical Experience Exchange Program.[2] The aim is to provide support for international training to promote the development of IBD patient care and research by enabling mentored observerships at established IBD centers.

Activities typically include participation in multidisciplinary clinics and case conferences; exposure to standardized monitoring and treat-to-target workflows; engagement with endoscopy, imaging, and pathology integration; introduction to clinical trial operations and data capture; and structured mentorship aimed at career development and local implementation. Beyond individual training, the program is intended to seed enduring collaborations and to enable evidence-based care pathways to be adapted to local resources.

Topic-focused satellite symposia. IOIBD and the IOIBD Institute convene Annual Satellite Symposia that complement the Annual General Meeting by disseminating advances in therapeutics, endpoints, and implementation. Representative topics include treat-to-target implementation (biomarker monitoring, endoscopic targets, and proactive disease control), optimizing sequencing and positioning of advanced therapies, management approaches for difficult-to-treat disease (including complex perianal Crohn's disease and pouch disorders), incorporation of nutrition and microbiome science into clinical care, evolving trial endpoints and regulatory expectations, and the increasing relevance of artificial intelligence and its role to optimize the assessment of treatment endpoints like endoscopic healing, histology, or transmural healing in clinical trials and clinical practice. These symposia are structured to highlight real-world implementation barriers and solutions and to identify research gaps for IOIBD clusters.

Empowering women in IBD leadership program

IOIBD launched an intensive leadership program to support the advancement of women in IBD, addressing persistent gaps in mentorship, visibility, and leadership opportunity.[2] Program elements, designed in collaboration with Pfizer, include a 9-month longitudinal structure with monthly mentor sessions, dedicated business coaching, and several live full-group sessions aligned with major international meetings. By combining structured leadership development with international networking, EMPOWHER aims to strengthen the pipeline of women leaders in IBD and reinforce IOIBD's commitment to excellence through diverse perspectives and inclusive global engagement.

Challenges and strategic evolution

IOIBD's model—selective membership, high standards, and consensus-driven outputs—has enabled sustained impact and requires thoughtful stewardship as the field expands. A continuing priority is to convert a wide breadth of expertise into focused, implementable insights. This is supported by clear prioritization within clusters, structured consensus methodologies, and a publications strategy that emphasizes actionable frameworks.

Another important consideration is achieving broad global engagement that is inclusive of developing nations while preserving IOIBD's defining characteristics: academic independence, methodological rigor, and a highly engaged membership. IOIBD supports global engagement through rotating meeting locations, a globalization cluster, competitive grants, and targeted educational and training programs. Furthermore, attention is given so that IOIBD recommendations and activities are relevant in low resource countries.

For maintaining active committed membership with relevant expertise and geographic diversity, there is a structured process for membership, in which candidates are nominated by current members and selected by the Nominations' Committee, which also oversees leadership succession.

Operational sustainability in a rapidly evolving therapeutic landscape is also an ongoing focus. The complementary roles of the Foundation (scientific mission) and the Institute (financing and educational logistics) provide a structure that supports sustainability while protecting credibility and independence.

Finally, in an era of globalization, expanding data streams, and the artificial intelligence revolution, IOIBD has begun to revisit its vision, mission, and strategy—positioning itself to be prepared for 2030 through a formal strategic planning process that includes collaborative efforts of its membership and updated vision and mission statements[2] (see call out box). This includes identifying how best to leverage real-world data, multi-omics, imaging, and remote monitoring; developing shared principles for responsible AI-enabled decision support; and promoting

equitable adoption of validated innovations across diverse healthcare settings. These strategic efforts align with IOIBD's mission to promote the health of people with IBD worldwide by setting direction for patient care, education, and research.

More specifically, IOIBD members have already begun to translate the AI agenda into concrete scientific initiatives. AI-assisted endoscopic assessment that includes deep-learning algorithms for real-time scoring of mucosal healing in ulcerative colitis and Crohn's disease is a recent example that addresses a critical need for standardized, reproducible endpoint capture in clinical trials and routine care. Recently, IOIBD developed consensus statements on AI/ML use for endoscopic and histologic endpoint assessment in IBD trials.[22] This IOIBD consensus supports the integration of AI/ML into central reading for IBD clinical trials to improve objectivity, efficiency, and consistency, while maintaining human oversight. Ongoing projects address validation, regulatory frameworks, and multimodal integration to enable broader adoption in both trials and clinical practice. These efforts exemplify how IOIBD expertise can accelerate the validation and adoption of AI-enabled tools as trial endpoints. Building on this, IOIBD is developing a broader framework for responsible AI integration in IBD, which includes: (1) leveraging large language models (LLMs) to harmonize and process unstructured patient-reported outcome data across multinational trials and diverse health systems, thereby improving comparability and scalability; (2) applying natural language processing to facilitate standardized data extraction from electronic health records for real-world evidence generation; and (3) developing shared governance principles for the deployment of AI-assisted clinical decision support, including explicit guidance on the risks of LLM hallucinations in the context of clinical guideline generation. Mitigating this requires rigorous validation frameworks, human expert review, and transparent disclosure of AI involvement in consensus processes; these are principles that IOIBD is committed to embedding into its AI strategy as the field moves toward data-intensive, precision-medicine approaches. Currently, IOIBD is evaluating AI-Enabled Endpoints for their routine use in clinical IBD trials to move toward precision medicine.

IOIBD Vision: The International Organization for the Study of Inflammatory Bowel Disease will, in the next decade, lead global efforts to improve the lives of patients with IBD and to advance the scientific understanding of IBD toward prevention and cures.

IOIBD Mission: The International Organization for the Study of Inflammatory Bowel Disease is an international group of clinicians and scientists who strive to advance IBD care, education, and research through global engagement, communication, and collaboration.

Conclusion and IOIBD 2030

IOIBD represents a distinctive and effective model of international cooperation in IBD. Through selective membership, structured governance, and continuously active thematic clusters, IOIBD has generated consensus outputs that have shaped therapeutic targets, standardized trial endpoints, elevated the role of patient-reported outcomes and disability, integrated nutrition as a core pillar of care, strengthened global epidemiology efforts, initiated IBD reclassification efforts, and supported clinical guidance during major disruptions such as the COVID-19 pandemic.

Looking toward 2030, IOIBD is revisiting its vision, mission, and strategic priorities to ensure continued leadership in advancing IBD science and improving outcomes for patients worldwide. Building on core principles of international collaboration, academic excellence, and independent consensus generation, IOIBD aims to further harness the collective expertise of its selective global membership. By embracing data-driven discovery, emerging digital and artificial intelligence-enabled approaches, and inclusive global engagement, IOIBD seeks to translate scientific innovation into clinically meaningful frameworks that shape research, therapeutic development, and patient-centered care in the decade ahead.

Declarations

Ethics approval and consent to participate

Not applicable.

Consent for publication

All authors have reviewed and consented to publication of its final version.

Author contributions

Iris Dotan: Conceptualization; Methodology; Supervision; Writing – original draft; Writing – review & editing.

Marla C. Dubinsky: Data curation; Writing – review & editing.

Corey A. Siegel: Methodology; Supervision; Writing – review & editing.

James Lindsay: Data curation; Writing – review & editing.

Dermot McGovern: Data curation; Writing – review & editing.

Matthieu Allez: Data curation; Writing – review & editing.

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Axel Dignass: Conceptualization; Data curation; Methodology; Supervision; Writing – review & editing.

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Competing interests

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and Johnson, Merck, Pfizer Inc., Prometheus Labs, Sanofi, Spyre, Takeda Pharmaceuticals. C.A.S.: Consultant/Advisory Board: AbbVie, Boomerang, Celltrion, Johnson and Johnson, Lilly, Napo Pharmaceuticals, Path Healthcare, Pfizer, Sanofi, Takeda, Trellus Health. Speaker for CME activities: AbbVie, Johnson & Johnson, Pfizer, Takeda. Grant support: AbbVie, Celltrion, Johnson & Johnson, Lilly, Pfizer, Takeda. J.Lindsay: consulting or speaker fees from AbbVie, Alpini, AstraZeneca, Bristol Myers Squibb, Celgene, Celltrion, Eli Lilly, Engitix, Ferring, Galapagos, Gilead, GlaxoSmithKline, Janssen, MSD, Orchard Therapeutics, Pfizer, Shattuck Labs, and Takeda; and has received investigator-led research grants from AbbVie, Gilead, and Takeda. D.M.: consultant for Mirador Therapeutics and holds patents related to anti-TL1A therapy in IBD. M.A.: financial support for research from Janssen, Takeda; consultant fees from AbbVie, Amgen, AstraZeneca, Biogen, Boehringer-Ingelheim, Bristol Myers Squibb, Celgene, Celsius, Celltrion, Egle Therapeutics, Endpoint Health, Ferring, Galapagos, IQVIA, Janssen, Lilly, MSD, Novartis, Owkin, Pfizer, Roche, Takeda, Tillots; and speaker fees from AbbVie, Galapagos, Genentech, Janssen, Lilly, Pfizer, Takeda. P.M.: has received honoraria for speaking or consultancy from AbbVie, Janssen, MSD, Ferring, Galapagos, Pfizer, Takeda, Baxter, Tillotts Pharma, and AstraZeneca. He has also received grants for research from Takeda, Janssen, and Ferring. J.Lewis: consulting or speaker fees from AbbVie, Alpini, AstraZeneca, Bristol Myers Squibb, Celgene, Celltrion, Eli Lilly, Engitix, Ferring, Galapagos, Gilead, GlaxoSmithKline, Janssen, MSD, Orchard Therapeutics, Pfizer, Shattuck Labs, and Takeda; and has received investigator-led research grants from AbbVie, Gilead, and Takeda. B.E.S.: honoraria for consulting from Aclaris Therapeutics, Adaxion Therapeutics, Adiso Therapeutics, Agomab Therapeutics, Alfasigma, Alimentiv, Amgen, AnaptysBio, AstraZeneca, Attovia Therapeutics, Biolojic Design, BiomeBank, Biora Therapeutics, Boehringer-Ingelheim, Caldera Therapeutics, Cellarity, Cytoki Pharma, Disc Medicine, Eil Lilly & Company, Ensho Therapeutics, Enthera, Enveda Biosciences, Equilium, Evommune, Ferring, Fzata, Galapagos, Genentech (Roche), Genesis Therapeutics, Gilead Sciences, GlaxoSmithKline, Globalport, Ignite Biomedical, Imedex, Imhotex, Immunitas Therapeutics, Immunyx Pharma, Kallyope, Kyowa Kirin, LifeMine Therapeutics, Merck & Co., Merck Sharp & Dohme, Microba, Microbiotica, Mirador Therapeutics, Mitsubishi Tanabe Pharma, Mobius Care, Morphic Therapeutic, MRM Health, Nexus Therapeutics, Nimbus Discovery, Novartis, Numab Therapeutics, Odyssey Therapeutics, ONO Pharmaceutical, OSE Immunotherapeutics, Palisade Bio, Pioneering Medicines Explorations, Prometheus Biosciences, Protagonist Therapeutics, Q32 Bio, Quotient Therapeutics, Rasayana Therapeutics, Recludix Pharma, Reistone Biopharma, Sanofi, Slate Bio, Sorriso Pharmaceuticals, Spyre Therapeutics, Synlogic, Target RWE, Televant, Teva, Thetis Pharmaceuticals, TR1X, Trex Bio, Union Therapeutics, VectivBio, Vedanta Biosciences, Vividion Therapeutics, Xencor; honoraria for consulting and speaking and other support from AbbVie, Abivax, Bristol Myers Squibb, Celltrion, Eli Lilly & Company, Genentech (Roche), J&J Innovative Medicine/Janssen, Merck & Company, Pfizer, Takeda, Teva; honoraria for consulting, stock and stock options from Ventyx Biosciences; and stock from Doximity. L.P.-B.: consulting AbbVie, Abivax, Adacyte, Alimentiv, Alfasigma, Amgen, Apini, Banook, BMS, Celltrion, Enthera, Ferring, Fresenius Kabi, Galapagos, Genentech, Gilead, Iterative Health, Janssen, Lilly, LifeMine, Medac, Morphic, MSD, Nordic Pharma, Novartis, Oncodesign Precision Medicine, ONO Pharma, OSE Immunotherapeuthics, Par' Immune, Pfizer, Prometheus, Roche, Roivant, Samsung, Sandoz, Sanofi, Sorriso, Spyre, Takeda, Teva, ThirtyfiveBio, Tillots, Vectivbio, Vedanta, Ventyx. Lecture Abbvie, Alfasigma, Amgen, Biogen, Celltrion, Ferring, Galapagos, Genentech, Gilead, Iterative Health, Janssen, Lilly, Medac, MSD, Nordic Pharma, Pfizer, Sandoz, Takeda, Tillots. S.D. consultancy fees from AbbVie, Alimentiv, Allergan, Amgen, Applied Molecular Transport, AstraZeneca, Athos Therapeutics, Biogen, Boehringer Ingelheim, Bristol Myers Squibb, Celgene, Celltrion, Dr Falk Pharma, Eli Lilly, Enthera, Ferring Pharmaceuticals Inc., Gilead, Hospira, Inotrem, Janssen, Johnson & Johnson, Morphic, MSD, Mundipharma, Mylan, Pfizer, Roche, Sandoz, Sublimity Therapeutics, Takeda, Teladoc Health, TiGenix, UCB Inc., Vial, Vifor. Lecture fees from Abbvie, Amgen, Ferring Pharmaceuticals Inc., Gilead, Janssen, Mylan, Pfizer, Takeda. V.A.: no relevant disclosures. G.K.: honoraria for speaking or consultancy from AbbVie, Janssen,

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Availability of data and materials

No new data were generated or analyzed in this review.

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